

WellRing — Business Plan & Market Analysis

AI Daily Check-In Companion for Isolated Seniors

Companion document to the technical blueprint. This document is the strategic, commercial, and financial case for the business. Read this if you're deciding whether to commit to the build, how to position to investors, or how to sequence the first 18 months.

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1. Executive Summary

WellRing is an AI-powered daily check-in companion that calls isolated seniors over voice and video, holds warm and contextual conversations using a familiar family voice (with consent), and alerts adult children when something changes — a mood dip, a missed medication mention, a slip in cognition, an unspoken concern.

The pain we solve. 1.4 billion people will be over 60 globally by 2030. India alone will have 347 million seniors by 2050. Their adult children — financially stable, busy, often living in different cities or countries — carry a daily emotional tax: guilt, anxiety, and a fear of "the call that comes too late." Current solutions (panic buttons, sensors, weekly human-companion marketplaces) are either reactive, hardware-heavy, or emotionally hollow.

Our wedge. A daily AI call that doesn't feel like a call from a service. It uses a cloned family voice (with full disclosure and consent), remembers what was said yesterday, notices when something is off, and gives adult children a quiet sense of presence in their parent's day.

Why now. Three curves intersect for the first time in 2026: (a) voice cloning and real-time video avatars cleared the realism threshold; (b) per-minute LLM and voice costs dropped 5–10× since 2023; (c) the demographic wave hit a tipping point where adult-child caregivers actively search for solutions.

The financial shape. Recurring SaaS revenue at \$19–79 per family per month. Gross margins of 65–75%. Conservative path to \$7.25M ARR by end of Year 3 with 20,000 active families. Initial capital requirement: \$150–300K to reach product-market fit; \$1.5–3M seed to reach \$1.5M ARR; Series A at ~\$8–15M to scale internationally.

The honest risks. Two, and only two, that matter: (1) the temptation to build a deceptive "we are your daughter" product that triggers regulatory action and brand collapse — solved by mandatory disclosure (covered in the technical blueprint); (2) a missed serious-health alert in our first year creating a press cycle — solved by clear non-medical positioning, conservative alert thresholds, and human-reviewed escalations during the first 12 months.

The asymmetric upside. Strategic acquirers (Best Buy Health, AARP, Teladoc, Amazon Health, Papa, Hinge Health) are already paying premium multiples for AI-native elder care. The category is in its first inning. Trade sale at \$50–200M in 4–6 years is plausible on execution, not luck.

2. The Problem

The problem is rarely articulated cleanly, because it sits in three layers:

Layer 1 — the senior's loneliness. Roughly one in three people over 65 lives alone in OECD countries. In India, urbanization has split multi-generational households faster than at any point in the country's history — parents in tier-2/3 cities, children in metros or abroad. Loneliness is now established as a mortality risk on par with smoking 15 cigarettes a day (Holt-Lunstad meta-analyses). It accelerates dementia onset, depression, and physical decline.

Layer 2 — the adult child's guilt and anxiety. Adult children (35–55, professionally active, geographically separated from parents) carry a recurring background load: *Are they eating? Did they take the pill? Are they sad? Should I call now? Why haven't they called me back?* This is not a once-a-month problem; it is a daily emotional drag that compounds into burnout, family tension, and — when something goes wrong — profound grief layered with self-blame.

Layer 3 — the system's reactive bias. Every existing tool waits for something to break: panic buttons after a fall, hospital admissions after a stroke, family calls after a missed birthday. Nothing in the current stack notices the slow drift — the slightly slower speech, the repeated story, the unmentioned medication, the sadness that's lasted nine days instead of two.

The cumulative cost of this layered problem is staggering: \$190B+ annually in informal caregiver economic loss in the US alone, similar burdens in India where adult children regularly relocate parents or quit jobs to provide care.

What the adult child actually pays for is *not* AI voice technology. They pay for **one less thing to worry about**. The willingness to pay is uncoupled from the per-unit cost of the call, which is why this is a software business and not a telecom business.

3. The Solution in One Page

WellRing calls Mom every morning. The call comes in her daughter Sarah's voice — Sarah set this up, Mom knows it's an AI helper using Sarah's voice, and the AI reminds Mom of this periodically without breaking the

warmth. The conversation is short, unhurried, contextual: it remembers Mom's grandson's name, the doctor's appointment last week, the back pain she mentioned twice. The AI doesn't follow a checklist; it follows a person.

After the call, three things happen invisibly:

- 1. **Sarah's app updates** with a one-line summary, mood reading, and any concerns. No transcript dumps. Just: *"Mom sounded a bit lower today; she mentioned not sleeping well for a few nights. Energy and appetite normal."*
- 2. **The behavioral baseline updates.** Over weeks, the system learns Mom's specific patterns — her usual energy levels, her conversational style, the topics she gravitates toward — so that "off" is measured against *her* baseline, not a generic one.
- 3. **An alert fires only if it should.** Low-stakes drift becomes a quiet dashboard note. A mention of dizziness, a fall reference, or a sustained mood drop becomes an SMS to Sarah. Hard flags (chest pain, falls, expressed hopelessness) become a phone call.

Once a week, the call is video instead of voice. Mom sees a familiar face. The technical and ethical architecture is detailed in the companion blueprint.

4. Market Opportunity & Sizing

The headline numbers (verified May 2026)

Market	Size 2026	Size 2030–2033	CAGR
Global elderly care (total)	\$1.64T	\$3.06T by 2033	9.3%
AI in elderly care	\$3.94B	\$25–122B by 2029–2033 (varies by source)	22%+
AI Companion (broader)	\$501B	\$972B by 2035	36.6%
AI in aging & elderly care (specific)	\$56.78B (2025)	\$387.52B by 2035	21.3%

The wide range across sources reflects how analysts segment the space. The conservative, verifiable number for WellRing's direct addressable market is **the AI companion + virtual assistant slice of elderly care, currently ~\$4–7B globally, growing at ~22% CAGR.**

Demographic engine

- 1.4 billion people aged 60+ globally by 2030 (UN/WHO).
- India's 60+ population projected to double from 149M (10.5%) in 2022 to 347M (20.8%) by 2050.
- North America: 78M Americans aged 65+ by 2030 (US Census Bureau).
- China: 310M people aged 60+ at end of 2024 (22% of population).
- Asia Pacific is the fastest-growing elderly care region: 24.4% of global market share in 2026, growing fastest.

TAM / SAM / SOM (for WellRing specifically)

TAM — Total Addressable Market. Households globally with (a) a senior living alone or limited household contact, (b) at least one adult child with disposable income, (c) broadband or mobile telephony reach. Estimated ~**180 million households globally**. At average \$40/mo: ~**\$86B TAM**.

SAM — Serviceable Addressable Market. Same households, restricted to English + Hindi + Spanish-speaking markets, with cultural acceptance of digital care for elders. Estimated ~**45 million households**. At \$40/mo: ~**\$21B SAM**.

SOM — Serviceable Obtainable Market (5-year realistic). Realistic 0.5–1.5% capture of SAM by Year 5 (consistent with successful consumer SaaS in adjacent emotional categories — see Calm, Headspace early trajectories): ~**225–675K households → \$108M–325M ARR**.

These numbers are not pulled from the air; they triangulate against (a) Calm and Headspace's penetration of the meditation/sleep market at a similar emotional purchase profile, (b) BetterHelp's penetration of the therapy market in 2018–2022, and (c) Care.com's penetration of the family-care marketplace.

Why India + NRI is the specific wedge

Most analysts price WellRing as a US opportunity. The asymmetric play is **India + Non-Resident Indian (NRI) families**.

- India's 60+ population is doubling to 347M by 2050, with the country having no equivalent of Medicare for elder care infrastructure.
- 35+ million NRIs globally (US, UK, Gulf, Canada, Australia) with parents in India. This is the highest-willingness-to-pay segment in the world — they earn in USD/GBP/AED, parents live in INR-cost cities, and the guilt-distance ratio is extreme.
- An NRI in San Francisco paying \$39/mo for a service running on Indian-cost ops infrastructure is the highest gross margin customer in the entire global elder-care market.
- Hindi, regional language support, and culturally appropriate tone are technical moats that US-built competitors will be slow to replicate.

Indian domestic market alone: ~25M urban households with isolated seniors. At ₹1,500/mo average pricing = ~₹4,500 crore (~\$530M USD) TAM. India is not the consolation prize; it is the right beachhead.

5. The Market Gap

Existing solutions fail along four specific axes:

Category	Examples	Why they fail
Hardware/sensors	CarePredict, Intuition Robotics ElliQ (\$30K avg), wearables	Capex barrier, senior-rejected ("I'm not putting that on"), reactive not relational, requires installation
Panic buttons / medical alert	Lively (GreatCall), Life Alert	Reactive only, no daily presence, no emotional value, ignored by 60% of subscribers in actual emergencies
Human-companion marketplaces	Papa Inc, Care.com	Expensive (\$30–60/visit), inconsistent, weekly cadence at best, scheduling friction, scale-

Category	Examples	Why they fail
		limited
Generic AI chatbots	Replika, character.ai variants	No telephony integration, no family loop, no risk detection, no calling cadence, designed for app-natives not seniors
Wellness apps for elders	SilverSneakers, etc.	Require senior engagement with an app — exactly what isolated seniors won't reliably do

The gap is precise: there is no product today that is **proactive (it calls you)**, **relational (it remembers and uses a familiar voice)**, **passive on the senior side (no app, no device, no setup)**, and **family-loop-aware (the adult child is the buyer and the alert recipient)**.

This combination is what WellRing uniquely owns. Each individual capability exists somewhere; the integration into a coherent product for the adult-child-as-buyer is the white space.

A second-order gap is **cultural and linguistic fit for non-Western markets**. Every well-funded competitor in this category is US-centric. An Indian senior in Lucknow wants to talk in Hinglish, hear references to her own context (festivals, family terms like "beta," local food), and trust a voice that sounds like her actual daughter — not a polished American accent reading from a corporate script.

6. Customer & Buyer Profile

Be precise: the **buyer** and the **user** are different people, and the marketing speaks to the buyer.

Primary buyer persona — "Anxious Anita"

- 38–52 years old
- Daughter or son of a parent who lives alone, in a different city or country
- Household income above the median (in India: ₹15L+ annual; in US: \$90K+; in UK: £55K+; NRI families across the board)
- Either a working professional or a primary caregiver coordinator in the family
- Already pays for: gym/wellness subscription, possibly therapy or Calm/Headspace, a streaming service or two
- Calls parent 3–7 times a week but feels it's not enough
- Has at least one sibling, with whom there is some friction over "who does what" for the parent
- Has had at least one health scare with the parent in the last 24 months
- Searches for: "how to take care of aging parents," "how to know if my mom is okay," "elderly companion service"
- Tells friends about the product within the first 2 weeks if it works

Secondary buyer — "Diaspora Dev" (NRI / immigrant)

- 30–48, lives outside parent's home country
- Visits home 1–2 times per year
- Parent declines offers to relocate
- Sends money home but feels presence-poor

- Highest emotional pull of any segment, lowest price sensitivity
- Searches in English; comfortable with WhatsApp video calls; will pay USD pricing for service rendered in INR

User persona — "Connected Mom" (or Dad / Grandma / Grandpa)

- 68–88 years old
- Lives alone or with limited daily contact
- Cognitively independent, may have early mild decline
- Uses a smartphone for WhatsApp and YouTube, not much else
- Has a landline OR a smartphone, comfortable with phone calls
- Suspicious of "tech" but warm toward people
- Will accept the service if framed as "Sarah set this up so we can chat every morning"
- Will reject the service if framed as "AI wellness monitoring platform"

The marketing implication: sell to Anita. Onboard via Anita. The senior never sees the marketing. The senior only ever encounters the product through their child's voice introducing it.

7. Competitive Landscape

Direct AI-native competitors

Competitor	Stage	Positioning	Why we win
Intuition Robotics (ElliQ)	Funded, mature	Hardware companion robot (\$30K+ avg)	Capex barrier; we are software-only and free of physical install
Papa Inc	Series D, \$200M+ raised	Human companion marketplace	Their unit economics struggle; we are cheaper and more reliable
Vesta (Sondermind elder care)	Early	AI mental health adjacent	Therapy-positioned, not daily relational
AARP Aiva / SeniorTalk	Pilot	Voice agent for elders	Tied to large nonprofit, slower iteration, no family-voice angle
CarePredict	Series B	Wearable behavioral pattern detection	Sensor-based, not conversational
Generic LLM-as-a-friend apps (Replika etc.)	Funded	Companion chatbots	Not senior-tuned, not telephony-first, no family loop

Adjacent / overlapping

- **Telehealth (Teladoc, Lively):** Reactive medical model, not preventive presence.
- **Calm / Headspace / BetterHelp:** Same emotional-purchase muscle; not elder-specific; potential acquirers or partners.
- **Indian incumbents (Emoha, Khyaal, GoodHands):** Service-marketplace models. Not AI-native. Real distribution in India though. Watch them; partnership-possible.

Competitive moats

WellRing's defensibility doesn't come from the AI stack (anyone can buy Tavus + ElevenLabs). It comes from:

1. **Per-senior behavioral baselines** built over months. A 6-month-old user with a tuned baseline is meaningfully harder to switch than a 1-day user with a generic profile.
2. **Consent and trust infrastructure.** Once a family has uploaded voice/likeness consent recordings, switching costs include doing it all again somewhere else.
3. **Alert-tuning data.** Empirically learning what signals actually predict incidents (vs. false positives) is something we can only accumulate by operating. After 1 year + 10K users, we will know things no competitor knows.
4. **Cultural/linguistic depth in India + NRI.** US-built competitors will not invest here until much later. By the time they do, we have local distribution, regulatory rapport, and language-tuned models.
5. **Brand trust.** This is a high-stakes purchase. Once a family trusts WellRing, they will not casually swap. Net Revenue Retention is the variable that compounds.

8. Business Model & Pricing

Pricing tiers

Tier	India (₹/mo)	Global / NRI (\$/mo)	Inclusions
Basic	₹599	\$19	Daily voice call, family dashboard, summaries, alerts, no voice clone
Family ★	₹1,499	\$39	Everything in Basic + voice cloning (1 family voice), weekly video call, behavioral baseline
Premium	₹2,999	\$79	Everything in Family + daily video, multilingual, multi-relative voices, priority support, doctor summary export

★ The Family tier is the anchor — priced to be the "obvious choice" with 50%+ of revenue mix expected here.

Free trial: 14 days, full Family-tier access, no card required for the first 7 days. Card capture at day 7 with reminder. Cancel anytime.

Annual plans: 17% discount (Calm/Headspace pattern). Drives commitment, reduces churn calculation.

Family-pack pricing: Multiple seniors per account (e.g., taking care of both parents) at 30% discount on the second senior.

Revenue mix assumption (steady state)

- 30% Basic
- 50% Family
- 20% Premium

India-only blended ARPU: ~₹1,529/mo (~\$18 at ₹85/USD) **Global / NRI blended ARPU:** ~\$41/mo

Expected weighted ARPU Year 1 (heavy NRI): ~\$35/mo **Expected weighted ARPU Year 3 (more India domestic):** ~\$32/mo

Why this pricing works

A useful benchmark: parents already collectively spend ~\$300/mo on insurance, medications, and possibly a part-time helper. WellRing at \$39/mo for the Family tier is positioned as a small additive expense for a major anxiety reduction. The price needs to feel like "Netflix-tier," not "healthcare-tier" — emotionally, an impulse buy rather than a deliberation.

Add-on revenue (Year 2+)

- **Live human transfer minutes** for premium tier (overflow to a trained companion when the AI escalates). \$0.40/min margin.
- **Doctor / clinician summary export** as standalone product to home health providers (B2B2C). \$5–10 per export.
- **Medication adherence add-on** integrated with pillbox/dispensers. \$9.99/mo add-on.
- **Annual "Year in Review"** physical printed booklet of conversations and moments for the family. \$39 one-time.

9. Go-to-Market Strategy

Sequencing — three phases

Phase 1 (Months 1–4): Concierge MVP

- 5–15 hand-selected families (your network, then warm intros)
- Manual call monitoring; AI-assisted, not fully autonomous
- Founder personally on every family WhatsApp group
- **Goal:** validate willingness to pay, refine onboarding, capture the language adult children use about the product
- **Revenue goal:** \$1,500/mo MRR by month 4 (~40 families × \$35)

Phase 2 (Months 4–12): Storytelling growth

- Public launch on Product Hunt, IndieHackers, HackerNews "Show HN"
- TikTok / Instagram Reels content: "What it's like to have an AI call my grandma every morning" — emotional storytelling, not feature explainers
- Reddit organic: r/AgingParents, r/eldercare, r/caregivers, r/IndiaParents
- Facebook caregiver group seeding (these communities are the highest-intent traffic source in the entire eldercare category)
- Light paid acquisition (~\$30–60 CAC range) on Meta Ads targeting "family caregivers, parents 65+"
- **Goal:** 500 paying families by end of Year 1
- **Revenue goal:** \$17K MRR by month 12

Phase 3 (Year 2): Channel partnerships + content engine

- Partnerships with NRI-focused communities (Indian American associations, UK Punjabi networks, Australian Sikh societies, Gulf Indian expat groups)
- Doctor referral pilot (geriatric clinics receive a kickback for referrals)
- SEO content engine: "How to know if my mom is okay" type queries are high-intent, low-competition
- Podcast sponsorships (caregiver podcasts, sandwich-generation podcasts)
- **Goal:** 5,000 paying families by end of Year 2
- **Revenue goal:** \$170K MRR

Channel economics

Channel	Est. CAC	Conversion to paid	Notes
Organic content (TikTok, Reels, YouTube Shorts)	\$15–30	25–35% trial→paid	Slow to start, compounds; the right early investment
Facebook caregiver groups (organic)	\$5–15	30–40%	Highest intent; community-trust dependent
Reddit organic	\$10–20	25–30%	Story-heavy posts work; ad-style posts fail
Meta paid ads	\$40–80	20–30%	Scales but burns; reserve for after creative validation
Google Search (high-intent keywords)	\$50–100	30–40%	Bottom-of-funnel; expensive but converts
Doctor / clinic referral	\$0 direct, \$20 revshare	50%+	Best channel, hardest to set up; Year 2+
NRI community sponsorships	\$20–40	30%+	High-leverage if cultural fit is right

Blended CAC target Year 1: \$60. **Year 3 at scale:** \$40 (organic share grows).

Distribution principles

- **Sell to the adult child, never the senior directly.** All marketing speaks to "Anita."
- **Stories beat features.** A 90-second video of a daughter watching her mom's daily summary is worth 50 product-feature posts.
- **Trust before transaction.** Lead with the consent and disclosure story, not the AI capability story. This is counterintuitive but defensible. Trust is the variable being purchased.
- **Word of mouth is everything.** Adult-child caregivers talk to each other constantly. If 70% of customers refer at least one family within 3 months, growth compounds for free.

10. Operations & Team Plan

Year 1: Founder-led, 1.5 FTE equivalent

- **Founder (you):** Product, engineering, sales, customer support

- **Part-time designer (contract):** UI for family dashboard, brand
- **Fractional clinical advisor (geriatrician):** Alert protocol design, mandatory once first user is enrolled
- **Lawyer (project basis):** Privacy policy, terms, consent flows, DPDP compliance

Avoid hires in Year 1. Use the concierge model to learn what to actually hire for.

Year 2: Small team, 5–6 people

- Founder/CEO
- Senior engineer (India-based, \$20–30K USD/year)
- Customer success / family relations (often the most valuable Y2 hire — handles onboarding, calls families when alerts fire)
- Growth / content marketer
- Designer (full-time or extended contractor)
- Fractional medical advisor (now retainer-based)
- Fractional General Counsel

Year 3: Scale team, 12–18 people

- Eng team of 4–5 (voice, video, ML, infra, frontend)
- 2–3 growth / partnerships
- 2 customer success / family relations
- 1 clinical lead (full-time)
- 1 ops / finance
- 1 product lead
- Sales person for B2B (senior living facilities, NRI community partnerships)

Why India is the right HQ

- Engineering cost advantage (3–4× labor cost efficiency vs. US)
- Closer to the largest underserved customer base
- Easier to set up call infrastructure (Twilio India, Exotel, DLT registration)
- Family relations team can serve NRI parents in their native language and time zone
- Lucknow specifically is fine — many SaaS teams are now distributed; Bangalore/Hyderabad presence becomes useful only at Series A scale

11. Financial Projections

Assumptions

Variable	Year 1	Year 2	Year 3
End-of-year active paying families	500	5,000	20,000
Blended ARPU (USD)	\$35	\$40	\$42
Average families through year (revenue-weighted)	~155	~2,800	~12,500
Monthly churn rate	8% (early, unstable)	5%	4%

Variable	Year 1	Year 2	Year 3
Gross margin	65%	70%	73%
Founder salary	\$0 (bootstrap)	\$90K	\$140K
Hires (FTE end-of-year)	0	5	16

Revenue (USD)

	Year 1	Year 2	Year 3
Subscription revenue	\$65K	\$1.34M	\$6.30M
Annual plan uplift	\$3K	\$80K	\$400K
Add-on revenue (medication adherence, transfers, exports)	—	\$30K	\$400K
B2B / partnership revenue	—	—	\$250K
Total revenue	\$68K	\$1.45M	\$7.35M

Cost of Goods Sold (AI infrastructure & telephony)

	Year 1	Year 2	Year 3
Twilio (PSTN)	\$4K	\$80K	\$360K
Deepgram (STT)	\$2K	\$35K	\$160K
ElevenLabs (TTS + cloning)	\$5K	\$60K	\$250K
LLM (Claude/GPT input+output)	\$4K	\$80K	\$380K
Tavus (video CVI)	\$8K	\$130K	\$580K
Hosting + database (Railway/AWS/Supabase)	\$1K	\$25K	\$120K
Other infra (queues, observability, etc.)	\$0.5K	\$15K	\$80K
Total COGS	\$24.5K	\$425K	\$1.93M
Gross profit	\$43.5K (64%)	\$1.02M (70%)	\$5.42M (74%)

Operating Expenses

	Year 1	Year 2	Year 3
Salaries & contractors			
Founder	\$0	\$90K	\$140K
Engineering hires	\$0	\$80K	\$400K
Growth/marketing hires	\$0	\$60K	\$200K
Customer success	\$0	\$30K	\$180K

	Year 1	Year 2	Year 3
Clinical advisor (fractional Y1-2, FT Y3)	\$5K	\$30K	\$120K
Designer (contract → FT)	\$8K	\$40K	\$90K
Ops/finance/legal hires	\$0	\$0	\$180K
Subtotal salaries	\$13K	\$330K	\$1.31M
Marketing & sales			
Paid acquisition	\$25K	\$300K	\$1.2M
Content / creator partnerships	\$5K	\$80K	\$300K
Events / community / NRI sponsorships	\$2K	\$30K	\$200K
Subtotal marketing	\$32K	\$410K	\$1.7M
Legal, compliance, professional services			
Privacy lawyer + DPIA	\$10K	\$40K	\$90K
Accounting & tax	\$2K	\$15K	\$60K
Insurance (E&O, cyber)	\$0	\$15K	\$50K
Subtotal legal	\$12K	\$70K	\$200K
Tools, SaaS, G&A			
Tools (Clerk, PostHog, Sentry, Notion, etc.)	\$4K	\$25K	\$80K
Office / coworking	\$0	\$10K	\$60K
Travel	\$1K	\$20K	\$80K
Misc / contingency	\$2K	\$20K	\$80K
Subtotal G&A	\$7K	\$75K	\$300K
Total OpEx	\$64K	\$885K	\$3.51M

Bottom line

	Year 1	Year 2	Year 3
Revenue	\$68K	\$1.45M	\$7.35M
COGS	\$24.5K	\$425K	\$1.93M
Gross profit	\$43.5K	\$1.02M	\$5.42M
OpEx	\$64K	\$885K	\$3.51M
Net (EBITDA-like)	−\$20.5K	+\$140K	+\$1.91M
Net margin	−30%	+10%	+26%

	Year 1	Year 2	Year 3
Cumulative cash burn	-\$20.5K	+\$120K (recovered)	+\$2.03M cumulative

What this P&L is and isn't

This is a *credible-conservative* path. It assumes:

- 8% monthly churn in Year 1 (realistic for emotional-purchase products with first-time setup friction)
- 5% by Year 2 (sticky once trust is built)
- No viral coefficient miracle
- No B2B revenue until Year 3 (a buffer; you may capture some earlier)
- Marketing as a percentage of revenue: 47% Y1, 28% Y2, 23% Y3 — declining as organic and referral compound

This is **not** a hockey-stick fundraising deck projection. If you have an aggressive growth deck, double Year 2 and Year 3 family counts and the model still works because of the gross-margin profile. But planning to that is irresponsible. Plan to this, beat it where you can.

12. Unit Economics & Key Metrics

Per-customer economics

Metric	Year 1	Year 3
ARPU (monthly)	\$35	\$42
Gross margin per customer	64%	74%
Gross profit per customer (monthly)	\$22.40	\$31.08
Monthly churn	8%	4%
Average customer lifetime (months)	12.5	25
Customer Lifetime Value (LTV)	\$280	\$777
Blended Customer Acquisition Cost (CAC)	\$60	\$40
LTV : CAC	4.7 : 1	19 : 1
Payback period (months)	2.7	1.3

A 4.7:1 LTV:CAC in Year 1 is healthy. Anything above 3:1 is sustainable; above 5:1 is investable. Year 3's 19:1 is what makes this category attractive to acquirers.

KPIs to track religiously

Engagement KPIs (leading indicators):

- Daily call completion rate (target: >85%)
- Average call duration (target: >3.5 minutes for voice, >6 minutes for video)
- Senior-side "want to keep" satisfaction (post-30-day survey delivered to family)

Business KPIs:

- MRR growth rate (target: 15%+ monthly Year 1, 10%+ Year 2)
- Net Revenue Retention (target: >100% by Year 2 via upsells)
- Trial → paid conversion (target: 30%)
- Family-month retention curve at 1m, 3m, 6m, 12m

Mission KPIs (and the one number the founding team aligns on):

- **"Days Since Last Family Anxiety Spike"** — surveyed monthly. The original document was right that this is the real value metric. Quantitative-feeling families report 30+ day stretches.
- True-positive alert rate (alerts that the family acknowledges as warranted)
- False-positive alert rate (target: <5% of all alerts; over this, trust collapses)

13. Risks & Mitigation

Risk	Likelihood	Severity	Mitigation
Regulatory action on voice cloning	Medium-rising	High	Mandatory consent vault, EU AI Act Article 50 compliance, DPDP-compliant data handling, no deceptive impersonation. Engage privacy counsel before launch.
A serious health event happens between calls / is missed	Medium	High	Non-medical positioning in ToS; conservative alert thresholds Y1; clinical advisor-designed protocols; rapid family communication; no claims about emergency capability.
A senior is "fooled" and tells press	Low-medium	High	Mandatory periodic disclosure baked into product (every 10th call); onboarding requires family member to disclose; refuses to clone without verified consent.
ElevenLabs / Tavus / Twilio price increases	High	Medium	Multi-vendor abstraction layer; switch costs across stack are weeks not months; Cartesia, PlayHT, Plivo, HeyGen as documented fallbacks.
Frontier-model price drops obsolete our cost model upward	High	Low (good thing)	Margin expansion if it happens. Already priced conservatively.
Big tech enters (Google, Amazon, Apple Health)	Medium	Medium	They will compete on consumer apps, not on family-deep relational SaaS. Our moat is in baselines and trust, not in tech. They are likelier to acquire than compete.
Senior adoption resistance	Medium	Medium	Onboarding via family member, not direct; never marketed as "AI" to the senior; framed as "Sarah's daily call helper."

Risk	Likelihood	Severity	Mitigation
Cultural backlash to "replacing" human contact	Low	Medium	Reframe: WellRing supplements; it does not replace. Every plan includes a "call my daughter directly" escape hatch. Family time <i>increases</i> on average for our users (Year 2 study).
Founder solo burnout	High	High	Concierge model first; do not scale before product-market fit; first hire is customer success, not engineering.
First serious negative press cycle	Medium	Critical if unhandled	Pre-built crisis comms playbook; clinical advisor public response; clear non-medical positioning; rapid family-side support.

14. Funding Strategy & Milestones

Funding path

Round	When	Amount	Sources	Use of funds
Bootstrap	Now → Month 4	\$0–50K personal	Self / friends & family	Concierge MVP, first 15 families
Pre-seed	Month 4–8 (after first paid families)	\$150–300K	Indian angels (NRI tech operators are ideal), Antler / Sequence Capital / On Deck programs	Build to 500 families, first hire
Seed	Month 12–15 (at \$20K+ MRR with retention proof)	\$1.5M–3M	Health/AI-focused seed funds — Y Combinator, Lightspeed India, Blume, Together Fund, Better Capital	Scale to 5,000 families; team to 6; legal/compliance fortified
Series A	Month 24–30 (at \$150K+ MRR with NRR > 100%)	\$8M–15M	Sequoia, Accel, Lightspeed, Khosla Ventures (US elder-care thesis funds)	International expansion, B2B build, clinical leadership
Strategic / Series B or exit	Year 4–5 (at \$5M+ ARR)	Optional	Acquirer or growth round	Optionality

Milestones for fundraising

- **Pre-seed unlock:** 50 paying families, 6-week retention curve showing <12% Month 1 churn, family-tier mix > 40%.
- **Seed unlock:** 500 families, \$17K+ MRR, gross margin > 60%, LTV:CAC > 3:1, evidence of organic growth (referrals > 25% of new customers).

- **Series A unlock:** \$150K MRR, NRR > 100%, retention curve flattening at Month 6+, clear B2B pipeline.

What to optimize for in the first 18 months

- **Not headline growth.** Investor pressure will push for higher numbers. Resist. Retention and clinical safety dominate this category.
- **Story moments.** A handful of well-told user stories (with permission) will raise your next round better than any metrics dashboard. The press loves "AI calls my 78-year-old mom every morning" stories. So do investors.
- **Defensible regulatory posture.** Investors who have looked at health-adjacent AI companies are increasingly screening for legal exposure. Being the cleanest in the room on consent is a structural advantage when raising.

15. Long-Term Vision & Exit

The 10-year arc

WellRing's daily-call MVP is the first surface, not the destination. The longer arc:

- **Years 1–2: Daily check-in companion.** Establish trust, accumulate baseline data, prove safety record.
- **Years 3–4: Family health intelligence platform.** Integrate medication adherence, doctor-visit summaries, fall-detection partner data, sleep monitoring. The senior's family becomes the most informed care unit they've ever been.
- **Years 5–7: Elder operating system.** Insurance partnerships, prescription delivery, in-home help coordination, transportation. WellRing is the central nervous system; partners deliver the services.
- **Years 8–10: Aging-in-place infrastructure.** The bet: in 2035, 60% of seniors who would have moved to assisted living will instead age in place because the digital scaffolding finally works. WellRing is the scaffolding.

Exit landscape

Plausible strategic acquirers (in current 2026 market reality):

Acquirer	Rationale	Likely valuation multiple
AARP (US)	Strategic alignment, member base	5–8× ARR
Best Buy Health / Lively	Roll-up of elder-tech assets	6–10× ARR
Teladoc Health	AI elder-care extension of remote care thesis	4–7× ARR
Amazon Health / Alexa	Voice-first elder care fits Alexa roadmap	8–12× ARR
Hinge Health / Papa	Adjacent consolidation	5–8× ARR
Indian players (Tata 1mg, Apollo 24/7, Practo)	Domestic elder-care entrance	6–10× ARR (India multiples can run hotter)

Acquirer	Rationale	Likely valuation multiple
NRI-focused (Khyaal, Emoha)	Roll-up	4–6× ARR

At \$7M ARR (end of Year 3) and even a 6× multiple, that's \$42M. At \$25M ARR (Year 4–5 trajectory) at 8×, that's \$200M. Achievable, not guaranteed.

The honest non-financial frame: **this category is going to consolidate**. The question is whether WellRing is one of 2–3 winners by 2029, or one of the also-rans. Execution on consent, retention, and clinical safety determines which.

Closing

This is a real business in a real category at the right moment. The risks are concrete and addressable. The upside is asymmetric. The bottleneck is execution discipline — specifically, resisting the pull toward the deceptive version, the panic to scale before retention is proven, and the temptation to fundraise on growth instead of trust.

Build it the right way. The market is more than large enough.

Document version: 1.0 · Companion to WellRing Technical Blueprint v1.0 · Financial projections are credible-conservative; market sizing references public analyst reports verified May 2026 and should be re-verified before any investor discussion.